

*Caring at its best***Patient ID Label**

or write name and number

Unit No.:

Name:

Address:

D.O.B.:

Ward:

Alcohol by volume and units

Strong Lager (9%)	– 440 ml	= 4 units
Beer / Lager (4.5%)	– 500 ml	= 2.2 units
Wine (12%)	– 175 ml	= 2.1 units
Spirits (40%)	– 25 ml	= 1 unit
Cider (4%) Litre		= 4 units
Strong White Cider (8%) Litre		= 8 units

Management of Alcohol Withdrawal Syndrome**Dependant drinking on screening - High Risk**

Any 2 of the following (tick):

- Presents with or has had previous withdrawal seizures ☐
- Previous severely agitated withdrawal (D.T.s) ☐
- High initial symptom score (GMAWS >8) (PTO) ☐

Yes**No****Exceptional Patient Group**Yes ☐ No ☐**Symptom Triggered Treatment**

as per GMAWS (PTO)

(Also consider Box 2 if exceptional group)

Please tick to confirm this treatment option ☐**Yes****No**

**Symptom
Triggered Treatment
on Lorazepam**
(see Box 2)

Fixed Dose Oral Diazepam**Initial 24 hours: 20 mg /6 hourly**

REDUCE as follows:

15mg Diazepam 6 hourly for 24 hours
 10mg Diazepam 6 hourly for 24 hours
 5mg Diazepam 6 hourly for 24 hours
 5mg Diazepam 12 hourly for 24 hours
 (Max 120 mg Diazepam in 24 hours before
 requesting senior medical review)

AND

- **Symptom Triggered Treatment, as per GMAWS (Box 1)**
- **If unable to take oral give 50% of the oral dose as IV**
- **In case of DTs please see Box 3**

Please consider **Pabrinex** for
all patients for the prevention
 and treatment of Wernicke's
 Encephalopathy.

Please see full Alcohol
 Withdrawal guidelines on INsite

10 mg Diazepam = 1 mg Lorazepam = 30 mg Chlordiazepoxide**Box 1 GMAWS**

Tremor
 Sweating
 Hallucination
 Orientation
 Agitation

(GMAWS PTO)

Box 2 Exceptional Patient Group:

1. Elderly >80 Y
2. Head injury
3. Encephalopathy
4. Jaundice
5. COPD / pneumonia
6. Low GCS

**Consider use of Lorazepam in a symptom-
 triggered fashion 1-2 mg (to a max of 12
 mg/24hr period before senior review)**

Box 3 Special Circumstances: Severe Withdrawal

(Delirium Tremens) with (agitation/aggression)

- Give 5-10 mg IV diazepam initially followed by a maximum of 10 mg every 5 mins, increasing to a maximum of 40 mg diazepam over 30 mins (assessing response) only to be given by FY2/higher grade or trained nurse
- Adjunct Haloperidol 2-10 mg PO/IM up to 18 mg in 24 hrs at 2 hourly intervals
- Flumazenil should be available on ward
- Seek senior review

Glasgow Modified Alcohol Withdrawal Scale (GMAWS)

Treatment options: GMAWS only ☐ GMAWS & Fixed Dose ☐

Date																				
Time																				
Tremor																				
0 No tremor																				
1 On movement																				
2 At rest																				
Sweating																				
0 No sweat visible																				
1 Moist																				
2 Drenching sweats																				
Hallucination																				
0 Not present																				
1 Dissuadable																				
2 Not dissuadable																				
Orientation																				
0 Orientated																				
1 Vague, detached																				
2 Disorientated, no contact																				
Agitation																				
0 Calm																				
1 Anxious																				
2 Panicky																				
Score																				
Treatment																				
Staff Signature																				

Score: (Do not use scoring tool if patient intoxicated, must be at least 8 hours since last drink)
0 Repeat score in 2 hours (discontinue after scoring on 4 consecutive occasions, except if less than 48hrs after last drink)
1-3 Give 10mg Diazepam: Repeat score in 2 hours
4-8 Give 20mg Diazepam: Repeat score in 1 hour
9-10 Give 20mg Diazepam: Repeat score in 1 hour and discuss wih medical staff regarding management of severe withdrawal as per guideline

All patients should have regular observations documented. Patients receiving high doses of Diazepam should be assessed regularly for over-sedation
Regular MEWS/SEWS - Frequency 1-4 hrs (GCS, Respiration Rate, Oxygen Saturation, Pulse, Blood Presssure)

Patients may require to be woken for continuing assessment
Co-existing illness may affect score: seek medical advice if in doubt
Fixed dosing and symptom triggered dosing must be no less than 1 hour apart